

CHAPTER 123

Atypical Melanocytic Nevus

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EPIDEMIOLOGY

The term *atypical melanocytic nevus* includes several clinical entities, such as common atypical dysplastic nevus, rare atypical Spitz or spitzoid nevus, atypical genital nevus, and other unusual melanocytic nevus. This chapter focuses on the common atypical/dysplastic nevus, which are relatively frequent melanocytic neoplasms.

Atypical nevus were first recognized as distinct pigmented lesions during the clinical assessment of melanoma-prone families. The nomenclature for these lesions includes **B-K moles** (named after the first two families described, whose surnames began with B and K), **familial atypical multiple mole and melanoma syndrome**, **atypical mole syndrome**, **Clark's nevus**, and **nevus with architectural disorder** (with varying degrees of melanocytic atypia). The terminology has been historically contentious. Currently, the term **dysplastic nevus (DN)** is more commonly used for both clinical and histologic diagnoses, and it is the author's preferred term. Thus, this term will be used throughout the chapter.

The usual clinical criteria for diagnosing a DN include two obligate features:

- Diameter of at least **5 mm** in one dimension
- **A prominent flat component**

Plus **two of the following three features**:

- Irregular, asymmetric outline

- Indistinct borders
- Variable pigmentation

(See Fig. 123-1)

Histologically, dysplastic nevi show:

- **Disordered or immature growth pattern**
- **Lymphocytic host response**
- **Random cytologic atypia in melanocytes**

(See Fig. 123-2)

Multiple studies have demonstrated that the clinical and histologic features of dysplastic nevi are associated with an increased risk of melanoma, particularly in individuals with familial melanoma syndromes.

ATYPICAL/DYSPLASTIC NEVI

AT A GLANCE

(Image placeholder: Clinical and dermoscopic appearance of dysplastic nevi illustrating irregular borders, color variation, and flat morphology)